

Table 2 *Caesarean section*

Case	Onset of fits	Duration of fits	Comment
8	3 days 5 hours	50 hours	Birthweight 2690 g. Apgar scores 3 and 5
19	4 days 6 hours	23 hours	Elective for cephalopelvic disproportion
21	3 days 9 hours	35 hours	Failed induction for toxæmia
23	5 days 2 hours	1 minute	Fibroid. Classical section
24	5 days 6 hours	3 hours	Elective for maternal oedema
29	3 days 12 hours	27 hours	Antepartum haemorrhage. Irritable, lethargic baby, feeding poorly
34	3 days 19 hours	12 hours	Android pelvis and breech presentation
37	5 days 10 hours	15 minutes	Failed induction, breech presentation
75	3 days 10 hours	12 hours	Unstable lie. Fetal distress
84	3 days	24 hours	Previous caesarean section
85	4 days 22 hours	13 hours	Birthweight 2460 g

Table 3 *Prematurity*

Case	Onset of fits	Duration of fits (hours)	Birthweight (g)	Gestation (weeks)	Comment
17	4 days 6 hours	11	2400	36½	—
32	4 days 8 hours	12	2490	34½	—
35	4 days 12 hours	10	2480	40	Induced
47	3 days 10 hours	22	2670	35	—
85	4 days 22 hours	13	2460	38	Caesarean section

63 were breast fed initially. They appeared to progress normally.

The fits

Fits were most often described as multifocal with jerking of limbs or face, often unilateral, and sometimes with apnoea or cyanosis. Their duration was from several seconds up to 2 or 3 minutes. Different patterns of fitting occurred at different times in the same case. No apparent precipitating factors were found but between fits the babies were sometimes noted to be jittery in response to stimulation. Neither apnoea alone nor a jittery response was regarded as a fit, although in some infants the seizures were heralded by apnoeic or cyanotic episodes. The frequency of fitting varied with some infants having fits every few minutes. One had 48 fits recorded within one day; others had only 1 attack, and one had 2 attacks separated by a 3-day interval.

Fitting lasted less than 24 hours in 55 (61%) cases and ranged from a single brief episode to one infant who had seizures for 15 days. Treatment did not consistently alter this duration of fitting (Table 4). Occasionally the end of fitting seemed to be related to administration of an anticonvulsant but other cases did not respond and some stopped within

24 hours without treatment with anticonvulsants. Thirty-seven were treated with phenobarbitone, 54 with phenytoin, 17 with diazepam by intravenous injection, and 10 with paraldehyde. Intravenous calcium was given in 15 cases and intravenous pyridoxine in 22 with no apparent effect.

Each baby remained in hospital until the fits stopped and he appeared to have recovered. There were no deaths. Treatment with anticonvulsants was continued in most infants after discharge from hospital.

Nearly all neonates remained in hospital for at least 5 days. Only one infant was known to have had his first fit after discharge from hospital (on the fifth day). It is possible that other infants had fits after discharge from hospital, but this is unlikely.

Investigations

Blood sugar was measured by Dextrostix in each infant and in at least 34 a laboratory blood glucose estimation was normal. Haemoglobin level and total and differential white cell counts were performed in nearly every case and were normal, as were the appearances of blood films. Skull x-ray films, serum sodium, potassium, and urea levels were all normal. Serum bilirubin levels were performed regularly if

Table 4 *Duration of fits and number of infants given particular treatment*

Duration (hours)	Phenobarbitone	Phenytoin	Calcium	Pyridoxine	Paraldehyde	Diazepam	Total cases
<12	11	15	3	3	0	2	22
12-24	21	20	7	9	5	6	33
25-36	11	9	2	5	4	4	17
>36	14	10	3	5	1	5	18